

TIGIT: 8 of 12 completed trials missed their primary endpoint

8 completed trials across 3 sponsors and 9 molecules ran to the end and missed their primary endpoint on the sponsor's own posted result; 2 met it. Only 3 of 60 closed trials were stopped early for efficacy or safety, which is why this class is read through its completed trials rather than a stop rate. Most completed trials post nothing readable, so the count is a floor.

8 of 12	3 / 9	5.0%	46.2%
completed trials with a readable primary result missed it · 2 met it	sponsors / molecules behind the misses	stopped early: 3 of 60 closed trials (95% CI 1.7%–13.7%)	of 130 trials have closed · 70 still open or unresolved

8 of 12

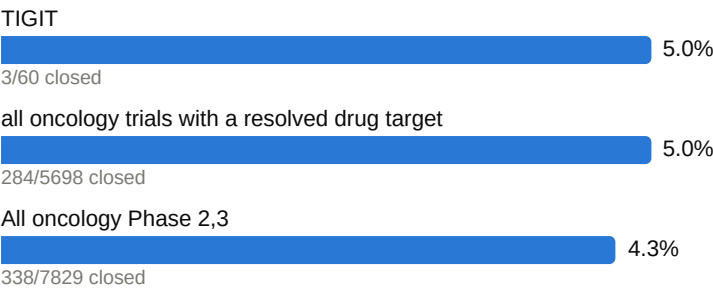
Completed and missed the primary endpoint. Of 40 completed trials in this segment, 12 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 8 missed and 2 met, 2 split across co-primary endpoints. Counted separately and never in the rate above; a floor, not a rate. 2 of the 29 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

The completed trials that missed

Trial	Ph	Sponsor	Experimental drugs	Started	What the posted result says
NCT05116202	2	Roche	TOBEMSTOMIG, ATEZOLIZUMAB, TIRAGOLUMAB	2022-02	Pathologic Response Rate (pRR) for Cohort 1 as Determined by Independent Pathologic Review — Difference in pRR 2.73 (95% CI -22.25 to 27.7): the interval includes no effect, so not significant.
NCT05014815	2	BeOne Medicines	OCIPERLIMAB	2021-11	Progression-free Survival (PFS) — one-sided p 0.4698, no threshold stated; 0.2 or above, so not significant at any one-sided threshold in use. Hazard Ratio (HR) 0.99 (95% CI 0.74 to 1.33). Method: Log Rank.
NCT04725188	2	Merck & Co.	PEMBROLIZUMAB, VIBOSTOLIMAB	2021-04	Progression Free Survival (PFS) Per Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) by Blinded Independent Central Review (BICR) Assessment — p 0.091 against 0.05: not significant. Hazard Ratio (HR) 0.77 (95% CI 0.53 to 1.13). Method: Regression, Cox.
NCT04732494	2	BeOne Medicines	OCIPERLIMAB	2021-03	Objective Response Rate (ORR) Assessed by the Investigator — p 0.2114 against the sponsor's stated threshold of 0.05: not significant. Risk Difference (RD) 9.9 (95% CI -5.4 to 25.3). Method: Cochran-Mantel-Haenszel.
NCT04665843	2	Roche	TIRAGOLUMAB	2021-03	Percentage of Participants With Confirmed Objective Response, as Determined by the Investigator Using Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST v1.1) — p 0.3886 against 0.05: not significant. Odds Ratio (OR) 1.57 (95% CI 0.56 to 4.41). Method: Cochran-Mantel-Haenszel.
NCT04513925	3	Roche	ATEZOLIZUMAB, TIRAGOLUMAB	2020-08	Progression-free Survival (PFS), as Assessed by an Independent Review Facility (IRF) in Programmed Death-ligand 1 (PD-L1) Positive Analysis Set (PPAS) — p 0.7586 against 0.05: not significant. Hazard Ratio (HR) 0.96 (95% CI 0.75 to 1.23). Method: Log Rank.

The 6 most recently started of 8 completed trials that missed. The diligence report lists every one, with the numbers or the sentence each verdict was read from.

How this compares



What is in this class: 129 distinct experimental drugs across 130 trials (antibody 124, small molecule 39, adc 14, unknown 9). Most tested: TIRAGOLUMAB (48), ATEZOLIZUMAB (34), PEMBROLIZUMAB (19), RILVEGOSTOMIG (17), VIBOSTOLIMAB (16). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 3 stops came from 3 sponsor-asset programmes across 3 sponsors; the largest (Roche) contributed 1; removing that programme's trials from both sides leaves 4.7%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 9 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 20.0%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2018–2020	0	22	0.0%	0.0%–14.9%
2021–2024	3	34	8.8%	3.0%–23.0%

Where the stops sit

Efficacy / futility: 3.
Sponsors with most stops: BeOne Medicines (1), Merck & Co. (1), Roche (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT04746924	3	BeOne Medicines ONC	OCIPERLIMAB	2025-05	Efficacy	This decision was conducted by the sponsor based on lack of efficacy and not driven by safety concerns.
NCT05226598	3	Merck & Co. MRK	PEMBROLIZUMAB, VIBOSTOLIMAB	2024-09	Efficacy	MK-7684A-007 was terminated early due to futility. A non-submission CSR, not intended to support any registration, was issued; therefore, an abbreviated CSR was generated.
NCT04308785	2	Roche	ATEZOLIZUMAB, TIRAGOLUMAB	2023-07	Efficacy	The sponsor's decision was based on the negative results of SKYSCRAPER-02.

Method

Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated).
Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits

Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 46.2% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. €99, delivered immediately: clinicaltrialfailures.com/asset-check

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxCClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.